

D455

Professional Nursing Role Transition

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A new graduate nurse on a busy surgical unit is working with an assistive personnel who has worked on the unit for eleven years and is well liked. The unit is short one nurse tonight. Which task is appropriate to delegate to the assistive personnel?

- A. Check whether a patient's pain improved after an IV opioid.
- B. Help a stable patient on post-op day 2 walk in the hall, as he has done twice today.
- C. Any task she feels comfortable with, given her eleven years of experience.
- D. Take vital signs on a patient who returned from surgery fifteen minutes ago.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which task must the RN keep rather than delegate to assistive personnel?

- A. The admission assessment of a patient arriving from the emergency department.
- B. Collecting a routine urine specimen.
- C. Feeding lunch to a patient with no swallowing problems.
- D. All care for any patient in isolation.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A charge nurse on a medical floor with a new electronic record system is making assignments. The LPN on shift transferred from a clinic last month and is eager to learn. Which patient is most appropriate to assign to the LPN?

- A. A patient just admitted with chest pain who needs an initial assessment.
- B. A patient whose care plan needs to be written today.
- C. A patient in the first 15 minutes of a blood transfusion.
- D. A patient with type 2 diabetes and stable glucose readings who is awaiting discharge tomorrow.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

After report on a unit where two nurses called in sick, the nurse has four patients. One family filed a complaint about parking yesterday. Which patient should the nurse see first?

- A. A patient 6 hours after thyroidectomy who has new high-pitched noisy breathing.
- B. A patient with diabetes whose pre-lunch glucose is 180 mg/dL.
- C. The patient whose family filed the parking complaint, who reports 8/10 incision pain on post-op day 1.
- D. A patient with COPD whose SpO2 is 89%, his usual baseline.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

Which of these patients can the nurse safely see last?

- A. A patient with a new onset of slurred speech.
- B. A patient with a heart rate of 136 after a fall.
- C. A patient with chronic kidney disease whose creatinine is at its usual elevated level.
- D. A patient on a PCA pump whose respiratory rate is 8.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A nurse on a night shift during a winter storm calls a provider about a patient with pneumonia whose oxygen saturation dropped from 95% to 88% over two hours. The provider has been on call for sixteen hours. Which statement belongs in the Assessment part of the SBAR?

- A. "I'm calling about her saturation, which dropped to 88% in the last two hours."
- B. "I'd like you to come evaluate her and consider a chest X-ray."
- C. "She was admitted two days ago with pneumonia and has a history of asthma."
- D. "I think her pneumonia is worsening and she may be developing respiratory failure."

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

A nurse gives a provider the Situation, Background, and Assessment, then pauses. What is still missing?

- A. A repeat of the Situation for emphasis.
- B. Nothing; SBAR is complete once the nurse has shared an assessment.
- C. The patient's full past medical history.
- D. A clear request, such as asking the provider to see the patient within 30 minutes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A provider known for excellent bedside manner orders a medication at ten times the usual adult dose. The nurse calls to clarify, and the provider says, "Just give it." The pharmacy is closing for the night. What should the nurse do next?

- A. Report the provider to the state board right away.
- B. Hold the medication and notify the charge nurse or nursing supervisor.
- C. Give the medication and document that the provider insisted.
- D. Give half the dose as a compromise before the pharmacy closes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

On a unit that just won a patient-experience award, a nurse skips barcode scanning because the scanner is slow and says "everyone does it." A wrong medication nearly reaches a patient before a second nurse catches it. Under just culture, how should the manager respond?

- A. Treat it as reckless behavior and begin discipline.
- B. Treat it as at-risk behavior: coach the nurse and fix why staff bypass the scanner.
- C. Discipline any nurse involved in any error, to show safety matters.
- D. Treat it as human error and simply console the nurse.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A patient scheduled for a colectomy, whose son drove four hours to be there, holds the consent form and asks the nurse, "What happens if something goes wrong during the surgery?" The surgeon is in another case. What should the nurse do?

- A. Hold the signing and ask the surgeon to answer the patient's questions before consent is signed.
- B. Tell the patient the consent is void because a question was asked.
- C. Explain the surgical risks so the patient can sign now.
- D. Have the patient sign now and ask the surgeon later.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A nurse manager who plays in a community orchestra tells staff that anyone whose charts pass the monthly documentation audit gets first choice of holiday shifts, and anyone who fails gets a written reminder. Which leadership style is this?

- A.** Laissez-faire.
- B.** Transformational.
- C.** Transactional.
- D.** Servant leadership.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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12. [TRANSFER — reversed stem / different variable]

Which behavior best shows transformational leadership by a charge nurse?

- A.** Giving a gift card to whoever has the fewest late charts.
- B.** Letting staff decide everything and stepping back entirely.
- C.** Sharing a vision of zero falls and mentoring staff to lead their own fall-prevention project.
- D.** Making all unit decisions alone to save time.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A new graduate nurse on a busy surgical unit is working with an assistive personnel ~~who has worked on the unit for eleven years and is well liked. The unit is short one nurse tonight.~~ Which task is appropriate to delegate to the assistive personnel?

Target word: a delegable task **Decides:** Stable patient plus routine, standardized task.

A	Confusion set	Evaluating a response to treatment is an RN responsibility.
B	Correct	Routine ambulation of a stable patient with a predictable response is delegable.
C	Camouflage	Experience does not expand the legal scope of delegation.
D	Dropped element	Vital signs are usually delegable, but a fresh post-op patient is not yet stable.

2. Answer A Concept 1 • transfer

Which task must the RN keep rather than delegate to assistive personnel?

Target word: non-delegable task — reverse direction **Decides:** Assessment belongs to the RN.

A	Correct	Initial assessment cannot be delegated.
B	Adjacent category	A standardized collection task that assistive personnel can do.
C	Inverted operation	This is a delegable task; the question asks what cannot be delegated.
D	Absolute language	Isolation changes precautions, not whether routine care can be delegated.

3. Answer D Concept 2

A charge nurse on a medical floor ~~with a new electronic record system~~ is making assignments. The LPN on shift ~~transferred from a clinic last month and is eager to learn.~~ Which patient is most appropriate to assign to the LPN?

Target word: LPN-appropriate patient **Decides:** Stable, predictable course.

A	Adjacent category	New admissions and initial assessments stay with the RN.
B	Dropped element	Care planning is an RN function even when the patient is stable.
C	Confusion set	The first minutes carry the highest reaction risk and need RN assessment.
D	Correct	Stable with a predictable outcome fits LPN scope.

4. Answer A Concept 3

After report ~~on a unit where two nurses called in sick,~~ the nurse has four patients. ~~One family filed a complaint about parking yesterday.~~ Which patient should the nurse see first?

Target word: first patient to see **Decides:** Only one has a new, acute airway problem.

A	Correct	New stridor signals airway obstruction—an acute, unexpected airway problem.
B	Adjacent category	Needs follow-up but is not immediately life-threatening.
C	Camouflage	Pain needs attention soon but does not outrank an airway threat; the complaint is irrelevant to priority.
D	Confusion set	A low-looking number that is expected and stable for this patient.

5. Answer C Concept 3 • transfer

Which of these patients can the nurse safely see last?

Target word: lowest priority — reverse direction **Decides:** Stable and expected findings.

A	Inverted operation	Possible stroke—seen first, not last.
B	Dropped element	New tachycardia after trauma may mean bleeding.
C	Correct	An expected, stable finding for a chronic condition.
D	Adjacent category	Respiratory depression is a breathing emergency.

6. Answer D Concept 4

A nurse on a night shift ~~during a winter storm~~ calls a provider about a patient with pneumonia whose oxygen saturation dropped from 95% to 88% over two hours. ~~The provider has been on call for sixteen hours.~~ Which statement belongs in the Assessment part of the SBAR?

Target word: the Assessment component **Decides:** Assessment = the nurse's interpretation of the problem.

A	Dropped element	That is the Situation; it states the change without interpreting it.
B	Confusion set	That is the Recommendation.
C	Adjacent category	That is Background.
D	Correct	States the nurse's clinical interpretation.

7. Answer D Concept 4 • transfer

A nurse gives a provider the Situation, Background, and Assessment, then pauses. What is still missing?

Target word: the missing SBAR element **Decides:** SBAR ends with a Recommendation.

A	Inverted operation	Circles back to the start instead of finishing with a request.
B	Absolute language	Without a recommendation, the call often ends with no action.
C	Adjacent category	Background has already been given; more history is not the missing step.
D	Correct	Recommendation states what the nurse needs.

8. Answer B Concept 5

A provider ~~known for excellent bedside manner~~ orders a medication at ten times the usual adult dose. The nurse calls to clarify, and the provider says, "Just give it." ~~The pharmacy is closing for the night.~~ What should the nurse do next?

Target word: next step with an unresolved unsafe order **Decides:** Clarification failed and harm is possible.

A	Adjacent category	External reporting skips the internal chain of command that protects the patient now.
B	Correct	The next step is escalation through the chain of command.
C	Inverted operation	The nurse is accountable for an unsafe dose regardless of who ordered it.
D	Camouflage	Responds to the closing pharmacy; the nurse cannot change a dose without an order.

9. Answer B Concept 6

On a unit ~~that just won a patient experience award~~, a nurse skips barcode scanning because the scanner is slow and says “everyone does it.” A wrong medication nearly reaches a patient before a second nurse catches it. Under just culture, how should the manager respond?

Target word: just-culture category and response **Decides:** A chosen shortcut with an unrecognized risk.

A	Adjacent category	Reckless means consciously ignoring a known, substantial risk; a common workaround is usually at-risk.
B	Correct	A normalized shortcut is at-risk behavior; coaching plus system repair fits.
C	Absolute language	Blanket punishment discourages reporting—the opposite of just culture.
D	Confusion set	Human error is an unintended slip; skipping a step on purpose is a choice.

10. Answer A Concept 7

A patient scheduled for a colectomy, ~~whose son drove four hours to be there~~, holds the consent form and asks the nurse, “What happens if something goes wrong during the surgery?” The surgeon is in another case. What should the nurse do?

Target word: nurse role in consent **Decides:** The patient has unanswered questions about risks.

A	Correct	The provider explains risks; the nurse ensures understanding before witnessing.
B	Absolute language	Questions are expected; they need answers, not cancellation.
C	Adjacent category	Explaining procedure risks is the surgeon’s responsibility.
D	Inverted operation	Consent must be informed before, not after, the signature.

11. Answer C Concept 8

A nurse manager ~~who plays in a community orchestra~~ tells staff that anyone whose charts pass the monthly documentation audit gets first choice of holiday shifts, and anyone who fails gets a written reminder. Which leadership style is this?

Target word: leadership style **Decides:** Rewards and corrections tied to meeting a standard.

A	Inverted operation	Laissez-faire gives little direction; this manager sets clear rules.
B	Confusion set	Transformational leaders inspire through vision and growth, not reward-for-target exchanges.
C	Correct	Exchanges rewards and consequences for performance.
D	Adjacent category	Servant leaders focus on meeting staff needs, not on audit-linked rewards.

12. Answer C Concept 8 · transfer

Which behavior best shows transformational leadership by a charge nurse?

Target word: behavior for a named style — reverse direction **Decides:** Shared vision and staff development.

A	Confusion set	A reward exchange is transactional.
B	Adjacent category	That is laissez-faire.
C	Correct	Inspires with a shared goal and develops others.
D	Inverted operation	Autocratic control is the opposite of developing others.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	11	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Delegation to assistive personnel	The RN may delegate routine, standardized tasks for stable patients (vital signs, hygiene, ambulation, feeding) but keeps assessment, teaching, evaluation, and care of unstable patients.	Delegating by task name alone without checking whether the patient is stable.	"Which task can the nurse delegate to the assistive personnel?"
2	Assigning to the LPN/LVN	LPNs/LVNs care for stable patients with predictable outcomes and collect data; the RN keeps initial assessments, care planning, and new or unstable patients.	Giving the newest or most complex patient to the LPN because the RN is busy.	Stable and predictable vs. new, changing, or unexpected.
3	Prioritizing which patient to see first	See first the patient with an actual, acute, unexpected airway, breathing, or circulation problem; findings expected for a chronic condition can wait.	Picking the most serious diagnosis or the loudest complaint instead of the unstable finding.	Four patients and "Whom should the nurse see first?"
4	SBAR communication	SBAR orders a handoff: Situation (what is happening now), Background (relevant history), Assessment (the nurse's interpretation), Recommendation (what the nurse is requesting).	Mixing up Assessment with Situation, or leaving out the specific request.	A nurse calling a provider about a change in condition.
5	Chain of command for unsafe orders	If an order seems unsafe, hold it and clarify with the prescriber; if still unresolved, escalate through the chain of command (charge nurse, supervisor) rather than carry it out.	Giving the medication because the provider insisted, or skipping straight to an outside agency.	A prescriber who will not change a questionable order.
6	Just culture and error reporting	Just culture separates human error (console and fix the system), at-risk behavior (coach), and reckless behavior (discipline); errors are reported so systems can improve.	Punishing every error the same way, or treating a common workaround as blameless.	A shortcut that "everyone on the unit takes."
7	Informed consent roles	The provider doing the procedure explains risks, benefits, and alternatives and obtains consent; the nurse witnesses the signature and notifies the provider if the patient has questions or seems not to understand.	The nurse explaining procedure risks, or having the patient sign now and ask questions later.	A patient with questions right before signing.

#	Concept	Rule	Trap	Tell
8	Transformational vs. transactional leadership	Transformational leaders inspire through shared vision and develop staff; transactional leaders manage through rewards, corrections, and rules; laissez-faire leaders give little direction.	Labeling any well-liked leader transformational, or any rule-based manager autocratic.	Vision and growth vs. rewards for meeting targets.

Course scope (public description): The Professional Nursing Role Transition course builds on the previous knowledge gained in all other nursing courses. The emphasis of this course is placed on the personal and professional strategies needed to make the transition from student to graduate nurse by highlighting the role and skills of bedside nurse leaders. The course will review content related to leadership and management, foundational nursing, advanced clinical, medical surgical, pediatric, women's, and mental health concepts. The students will complete a professional portfolio that showcases their accomplishments, knowledge, and skills throughout the program. A variety of populations and settings are used in the experiential learning components of this course. Co-requisite: D454